



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : نداء كمال السيد علي
المحافظة : الفيوم
الحى / المدينة : سنورس
رقم البطاقة : 27306210100125 رقم التليفون : 01121284067
العنوان : خ
العمر (تاريخ الميلاد) : 21-06-1973 المهنة : ربه منزل
تاريخ تسجيل الإستمارة : 21-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام ماموجرام وسونار علي الثديين [10888] قسم العلاج : الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة: -----
الوظيفة: -----
الوظيفة: -----
التوقيع: -----
التوقيع: -----
التوقيع: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. / ----- / أ.د. / ----- / د. / ----- / د. / -----

قرار اللجنة

التشخيص النهائي : -----
خطة العلاج : -----
المستشفى : -----
جهة العلاج : -----
ملاحظات : -----
قسم : -----
نفقات : -----

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: نداء كمال السيد علي National ID: 27306210100125	
--	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Mammography and breast ultrasound
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

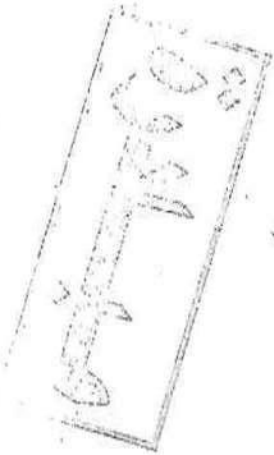
IM5331008



جمهورية مصر العربية
بالتة تقوى الشخصية

لنام
كمال السيد على شحاته
باسون
مركز القناطر الخيرية - القنوية

٢٧٣٠٩٢١٠١٠٠١٢٥



Handwritten signature

ERM036745176



Handwritten signature

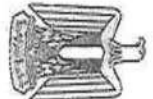


٢٠٢١/٠٥ ٢٧٣٠٩٢١٠١٠٠١٢٥

حامله على داليم تجارة

انثى

محمدة سليمان عبدالرازق احمد
البطانة سارية حتى ٢٠٢١/٠٥/٠٩



Handwritten signature and date ٢٠٢١/٠٥/٠٩

03/25/2026

MDT FORM

Name:	NEDA Kamel El-Sayed Ali	MIRN:	3191
Age:	53 Y	ID:	1289-2025
Menopausal Status:	premen	Phone:	0112128406
NRS SCORE:	PS 1	Marital Status:	
BMI:		Family History:	
Co Morbidity:		Reason Of MDT:	

Clinical Data:

L

R

BX:

ER: PR: HER2: K167:

Sonomammography:

Mets Workup:

History:

T N M

Recommendations:

MDT Outcome & Plan

In view of cardiac impairment, for:

- Cardiological consultation
- Start Neoadjuvant ET (Femara) with close monitoring of response...then go for Rt MRM.

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	NUTRITION	PSYCHIATRIC

Patient Name : الأستاذة / نداء كمال السيد على	Request No. : 13005978
Patient No. : 27306210100125	Request Date : 10/04/2025
Path No. : 1268/25	Verification Date : 04/05/2025
Sex : Female	Financial Category : أورام دار السلام - مبادره
Age : 51 Y	Department : Out Patient
Referring Doc :	Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Right breast malignant looking mass.
Nature of Specimen: Tru-cut biopsy

Gross Examination:

- Five grayish white tissue cores up to 1 cm, totally submitted.
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, Her2, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), Ki67 clone (30-9) and SISH.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
- * Ductal carcinoma in situ: Focal
- * Perineural invasion: No
- * Angioinvasion: No
- * Microcalcifications: Positive
- * Additional pathologic findings: Wide area of necrosis
- * IPT (Positive control showed appropriate reactivity):
 - E-Cadherin: Positive
 - ER: Strong nuclear reaction in 90% of tumor cells (score 8/8).
 - PR: Weak nuclear reaction in 30% of tumor cells (score 4/8).
 - Her2: Moderate complete membrane reaction in >10% of tumor cells (score 2+).
 - Ki67: 50% nuclear reaction of tumor cells.
 - SISH: Not amplified.
- Average Her2 copy number : 1.1
- Her2/CEP17 ratio:0.9

Verified by :
DR.Menna Nabil

Printing Date : 31/05/2025 10:16:44

Page 1 of 3

Printing By : Mrs.Mayada Hegazy

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

CAP
ACCREDITED

Patient Name : الأستاذة / نداء كمال السيد على
Patient No. : 27306210100125
Path No. : 1268/25
Sex : Female
Age : 51 Y
Referring Doc :

Request No. : 13005978
Request Date : 10/04/2025
Verification Date : 04/05/2025
Financial Category : أورام دار السلام - مبادره
Department : Out Patient
Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Right breast malignant looking mass.
Nature of Specimen: Tru-cut biopsy

Gross Examination:

- Five grayish white tissue cores up to 1 cm, totally submitted.
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, Her2, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic Examination:

- * Histological type: Invasive duct carcinoma
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II.
- * Ductal carcinoma in situ: Focal
- * Perineural invasion: No
- * Angioinvasion: No
- * Microcalcifications: Positive
- * Additional pathologic findings: Wide area of necrosis
- * IPT (Positive control showed appropriate reactivity):
- E-Cadherin: Positive
- ER: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- PR: Weak nuclear reaction in 30% of tumor cells (score 4/8).
- Her2: Moderate complete membrane reaction in >10% of tumor cells (score 2+).
- Ki67: 50% nuclear reaction of tumor cells.

Diagnosis:

Right breast mass, Tru-cut biopsy:
Invasive duct carcinoma, Grade II with focal DCIS.

Verified by :
DR.Menna Nabil



Patient Name	: الأستاذة / نداء كمال السيد على	Request No.	: 13005978
Patient No.	: 27306210100125	Request Date	: 10/04/2025
Path No.	: 1268/25	Verification Date	: 04/05/2025
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 51 Y	Department	: Out Patient
Referring Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

IPT:

- E-Cadherin: Positive
- ER: Positive (score 8/8).
- PR: Positive (score 4/8).
- Her2: Equivocal (score 2+) for SISH.
- Ki67: Positive in 50% of tumor cells.

• Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

• Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature

Dr. Rana Fathy

Dr. Marwa Mahmoud

Verified by :

DR.Menna Nabil





Patient Name: Nedaa Kamal Al Sayed Ali Shehata	Date: 04/02/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 51 Year(s)
Medical ID: 27306210100125	Pat. ID: 97428

Combined bilateral breast mammography and complementary ultrasound

Indication:

left breast palpable lump.

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Right breast shows UOQ spiculated irregular mass lesion, of high density.
- **Complementary ultrasound revealed:**
- Right breast irregular spiculated hypoechoic mass lesion, with subtle vascularity detected within on color Doppler, seen at 11 to 1 o'clock position reaching the nipple, 0.3cm from the overlying skin, measuring 2.7x4.8cm, consistent to the palpable lump.
- Few Pathological right axillary lymph nodes level I are seen with muffled hila, measuring 1.1x1.6cm, with cortical thickness reaching 5mm
- Single aright axillary level II interpectoral rounded LN measured 6x8mm
- Free right supra and infraclavicular regions.
- No left breast lesions.
- Normal skin thickness, and nipple-areola complex of the left breast.
- Nonspecific left axillary LN.

Opinion:

- Right breast 11 to 1 o'clock malignant looking mass lesion (BIRADS 5) for histopathological correlation.
- Pathological right axillary LNs level I&II.
- Benign findings left breast (BIRADS 2)

Yours truly

Signature Dr. basma Mohamed

04/02/2025 3:26 PM

for MACT followed by mastectomy
if not metastatic

Patient ID: 90514	Patient Name English: Nedaa Kamal Alsid Ali Shehatah
Age: ٥٢ سنة, ٥ شهر	2025/12/08 01:02:55 PM :Report Date

Women's Health Outreach Program
Bilateral full field Digital Mammography

Clinical summary: right breast pathological proven IDC

Technique

Medio-lateral oblique and CC views & High resolution ultrasound

Detailed Findings

Left breast

- Scattered fibroglandular densities (ACR B)
- No masses, asymmetric densities, distortion or suspicious clusters of microcalcifications.
- **Ultrasound** demonstrated no solid or cystic lesions.
- Normal skin and nipple/areolar complex.
- Non-specific axillary LNs.

Right breast

- Scattered fibroglandular densities (ACR B)
- Spiculated dense lesion seen occupying the whole breast with internal microcalcifications, measures 9x7 cm, complementary ultrasound showed superficial spiculated hypoechoic solid lesion with surrounding desmoplastic reaction seen about 1.5 from the nipple.
- Diffuse skin thickening reaching 4 mm
- Pathological axillary lymph nodes (level I), more than three, with increased cortical thickness and lost hilum the largest measures 2x1.8 cm, another Pathological axillary lymph node (level II) measures 2.5x1.8 cm

Impression:

Left breast (BI-RADS 1): normal study

Right breast (BIRADS 6): large pathological proven malignant lesion occupy the whole breast, with pathological axillary lymph nodes (level I, II)

Best regards,
Dr. Sara gamal , MSc

Patient Name: Nedaa Kamal Al Sayed Ali Shehata	Date : 10/11/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 52 Year(s)
Medical ID: 1288/2025	Pat. ID: 97428

CT chest, abdomen and pelvis with contrast

Previous Exams:

- This is the baseline metastatic work-up, non-comparative study for a patient with known history for right sided breast cancer recently discovered.

FINDINGS:

Chest:

- Right breast central/ outer quadrant necrotic mass lesion measures 5 cm associated with multiple pathologically enlarged right axillary lymph nodes (2 cm; the largest) and right sub-pectoral lymph node (1.7 cm) ... **for sono-mammographic correlation.**
- Clear both pulmonary parenchymal lung fields with no focal mass lesions, nodules or consolidative patches
- The trachea is central in position.
- No mediastinal mass lesions, normal enhancement of the mediastinal vessels.
- No suspicious mediastinal or hilar lymph nodes.
- No pleural thickening with no pleural collections identified.
- Normal cardiac size. No pericardial effusion.

Abdomen:

- Tiny subcapsular right hepatic lobe segment IV hypo-enhancing focal hepatic nodule measures 8 mm ... **for triphasic MRI assessment/ tissue biopsy and histopathological correlation.**
 - Otherwise; the liver shows normal size and density with no enhancing focal hepatic lesions.
- Normal size and density for the spleen with no enhancing focal mass lesions.
- The pancreas and supra renal glands bilaterally are unremarkable.
- Both kidneys show normal size with no suspicious mass lesions. Intact parenchymal thickness, no renal stones or back pressure changes,
- Scans through the pelvis show no mass lesions.
- No suspicious enlarged abdominal or pelvic lymph nodes. No ascites.

Opinion:

- Unremarkable chest study with contrast.
- Tiny subcapsular right hepatic lobe segment IV hypo-enhancing hepatic nodule ... **for triphasic MRI assessment/ tissue biopsy and histopathological correlation.**
- Right breast central and outer quadrant mass lesion with metastatic ipsilateral axillary and subpectoral lymph nodes... **for sono-mammographic correlation.**

Thanks for referral;
Dr. Reham Mohamed, MS., FRCR.

to be evaluated at the MDT

Name: Nedaa Kamal Al Sayed Ali Shehata
Referred by: Prof.

ID: 4961305
Date: 09/12/2025

Neck ultrasound

- Right thyroid lobe is surgically removed with clear surgical bed.
- Left thyroid lobe is normal in size showing homogenous echopattern with average vascularity, no nodules or cysts.
 - Left lobe = 15x13x22 mm.
- Both parotid glands and both submandibular glands are normal in size with homogenous echopattern and average vascularity.
- Central trachea.
- Both common carotid arteries are patent.
- Both internal jugular veins are patent with no thrombosis.
- No enlarged cervical lymphadenopathy.

Opinion:

- Status post ? right hemithyroidectomy with no evidence of residual or recurrence for surgical history correlation.

Much obliged,
Dr. Bahaa El-Din Sayed, MD